

SECTION 14

TRC Calls to Action 18–24

Seven health Calls. December 2015. Ten years. Yellowhead Institute's September 2025 characterization: "Canada's health systems have perfected performative reconciliation." The Calls, the status, and what they require of Alberta in relation to the disabled Indigenous Albertans this report concerns

The Truth and Reconciliation Commission of Canada issued 94 Calls to Action on December 15, 2015. Seven of them — Calls 18 through 24 — address health directly. Ten years later, the Yellowhead Institute's September 2025 Braiding Accountability review concluded that Canada's health systems have perfected performative reconciliation. Symbolic gestures have multiplied. Structural change has not. The seven Calls remain largely open. The population they were written to protect — the documented population of this report — continues to die nineteen years sooner than their non-Indigenous neighbours."

This section documents the seven health-related Calls to Action of the Truth and Reconciliation Commission of Canada, their status as of April 2026, and their direct application to the disabled Indigenous Albertan population this report concerns. The section builds on Section 1 (residential schools and the direct causal framing), Section 7 (the dual healthcare system), Section 11 (MMIWG2S+ and the 231 Calls for Justice), and Section 12 (UNDRIP obligations). What Section 14 adds is the specific, named, numbered reconciliation framework the federal and provincial governments of Canada have formally accepted as the measure of their own progress — and the documented record of that progress as of the ten-year mark.

Four findings structure the section. First, that the TRC Calls to Action 18 through 24 are the specific health reconciliation framework applicable to this report's subject matter, and each of the seven Calls connects directly to sections of this report. Second, that the Yellowhead Institute's September 2025 ten-year review of the health Calls found that implementation has been predominantly symbolic, with structural change absent and system-wide transformation lacking. Third, that Alberta specifically has, among provincial jurisdictions, made partial written acknowledgment on Call 18 but has not produced the structural follow-through — and that the July 1, 2026 ADAP transition and December 2025 Bill 12 both operate in explicit tension with Calls 18 and 20 by reducing the health-related supports available to a population the Calls direct governments to improve conditions for. Fourth, that for a disabled Indigenous Albertan in April 2026, the TRC Calls to Action 18 through 24 provide a binding moral framework — accepted in

principle by all relevant governments — whose operational failure is itself documented evidence of the pattern this entire report has been tracing.

Part 1: The seven Calls, in their own words

The following are the seven health Calls to Action in the exact words of the Truth and Reconciliation Commission of Canada's Final Report, released December 15, 2015. They are listed verbatim because the specificity of the Commission's language is itself significant. Each Call identifies specific actors, specific obligations, and specific outcomes. None of them is aspirational. All of them are directed.

Call to Action 18

We call upon the federal, provincial, territorial, and Aboriginal governments to acknowledge that the current state of Aboriginal health in Canada is a direct result of previous Canadian government policies, including residential schools, and to recognize and implement the health-care rights of Aboriginal people as identified in international law, constitutional law, and under the Treaties.

Call to Action 19

We call upon the federal government, in consultation with Aboriginal peoples, to establish measurable goals to identify and close the gaps in health outcomes between Aboriginal and non-Aboriginal communities, and to publish annual progress reports and assess long-term trends. Such efforts would focus on indicators such as: infant mortality, maternal health, suicide, mental health, addictions, life expectancy, birth rates, infant and child health issues, chronic diseases, illness and injury incidence, and the availability of appropriate health services.

Call to Action 20

In order to address the jurisdictional disputes concerning Aboriginal people who do not reside on reserves, we call upon the federal government to recognize, respect, and address the distinct health needs of the Métis, Inuit, and off-reserve Aboriginal peoples.

Call to Action 21

We call upon the federal government to provide sustainable funding for existing and new Aboriginal healing centres to address the physical, mental, emotional, and spiritual harms caused by residential schools, and to ensure that the funding of healing centres in Nunavut and the Northwest Territories is a priority.

Call to Action 22

We call upon those who can effect change within the Canadian health-care system to recognize the value of Aboriginal healing practices and use them in the treatment of Aboriginal patients in collaboration with Aboriginal healers and Elders where requested by Aboriginal patients.

Call to Action 23

We call upon all levels of government to: i. Increase the number of Aboriginal professionals working in the health-care field. ii. Ensure the retention of Aboriginal health-care providers in Aboriginal communities. iii. Provide cultural competency training for all healthcare professionals.

Call to Action 24

We call upon medical and nursing schools in Canada to require all students to take a course dealing with Aboriginal health issues, including the history and legacy of residential schools, the United Nations Declaration on the Rights of Indigenous Peoples, Treaties and Aboriginal rights, and Indigenous teachings and practices. This will require skills-based training in intercultural competency, conflict resolution, human rights, and anti-racism.

The seven Calls, read together, describe a structural reform framework. Acknowledgment of causation (Call 18). Measurement of gaps (Call 19). Jurisdictional responsibility for the off-reserve population (Call 20). Sustainable funding for Indigenous-led healing infrastructure (Call 21). Integration of Indigenous healing practices into Canadian healthcare (Call 22). Workforce reform to increase Indigenous representation in healthcare delivery (Call 23). Education reform to embed Indigenous health, UNDRIP, Treaties, and anti-racism in the training of every future Canadian physician and nurse (Call 24). Any one of these Calls, fully implemented, would constitute substantial reform. Together, they constitute the transformation the Commission considered necessary for health reconciliation.

Part 2: Status at ten years

The Yellowhead Institute 2025 review

In September 2025, the Yellowhead Institute published *Braiding Accountability: A Ten-Year Review of the TRC's Healthcare Calls to Action*. The Yellowhead Institute is an Indigenous-led research and education centre based at Toronto Metropolitan University, with an established record of evaluating Canadian government compliance with reconciliation commitments. The ten-year review applied the Braiding Framework for Health Accountability, developed from the Cultural Responsiveness Framework, to assess progress along three interrelated strands: restoring Indigenous wellness systems, establishing ethical space for engagement, and transforming mainstream service delivery.

The ten-year review's central finding: "Canada's health systems have perfected performative reconciliation." The Yellowhead review found that Indigenous-led healing centres remain underfunded, jurisdictional disputes continue, and Indigenous health education is inconsistently embedded in medical and nursing curricula. Although promising initiatives exist, system-wide transformation is lacking. The review's characterization is direct: "symbolic gestures divorced from structural accountability do not constitute reconciliation."

*Canada's health systems have perfected performative reconciliation. —
Yellowhead Institute, Braiding Accountability: A Ten-Year Review of the
TRC's Healthcare Calls to Action, September 2025"*

The CBC Beyond 94 tracking

CBC News established Beyond 94 in 2018 as a public tracker of the 94 Calls to Action. As of May 2025, the CBC Beyond 94 assessment marked 15 of the 94 Calls as "complete," 24 as "in progress," and the remainder as either proposed or not yet started. None of the seven health Calls (18 through 24) was marked complete. Implementation of the health Calls specifically has been partial, uneven across jurisdictions, and characterized by government acknowledgment statements without the structural follow-through the Calls require.

The Indigenous Watchdog assessment of Call 18

The Indigenous Watchdog tracking of Call to Action 18 provides jurisdiction-specific detail. On the first part of Call 18 — acknowledgment that the current state of Aboriginal health is a direct result of previous government policies — only the federal government, British Columbia, Alberta, Manitoba, Ontario, Yukon, and the Northwest Territories have explicitly made an official policy statement to that effect. On the second part — recognizing and implementing the health-care rights of Aboriginal people as identified in international law, constitutional law, and under the Treaties — only the federal government, British Columbia, and the Northwest Territories have operationalized explicit policy action.

Alberta's position, as documented in the Indigenous Watchdog tracking, is: acknowledgment of Part 1 of Call 18 on the historical causation, but no operational implementation of Part 2 on the healthcare rights of Aboriginal people under international, constitutional, and treaty law. The acknowledgment without implementation pattern is precisely the "performative reconciliation" characterization the Yellowhead Institute named in its September 2025 review.

Part 3: Call-by-Call status in Alberta context

Call 18: Acknowledgment and implementation of health-care rights

Status in Alberta: The Government of Alberta has, across multiple administrations, made written acknowledgment statements recognizing that the current state of Aboriginal health is shaped by

historical Canadian government policies. The implementation of the health-care rights of Aboriginal people under international law (UNDRIP — Section 12 of this report), constitutional law (section 35 and Treaty — Section 13), and under the Treaties (medicine chest clause — Section 7) remains unoperationalized in Alberta provincial practice.

Direct tension with ADAP: Call 18 directs governments to implement the healthcare rights of Aboriginal people under the Treaties. The Treaty 6 medicine chest clause established a federal obligation to provide healthcare. The NIHB program operationalizes that obligation in narrowed form (Section 7). The July 1, 2026 ADAP transition reduces provincial disability supports that interact with federal NIHB in the dual system documented in Section 7. The transition therefore operates in tension with the spirit of Call 18, which directs that treaty health rights be recognized and implemented — not reduced.

Call 19: Measurable goals and annual progress reports on health outcome gaps

Status: Call 19 directed the federal government, in consultation with Aboriginal peoples, to establish measurable goals on gaps in health outcomes and to publish annual progress reports. Indigenous Services Canada has developed an indicator framework; annual public reporting has been uneven. Disaggregated data on indicators including infant mortality, maternal health, suicide, mental health, life expectancy (Section 2 of this report documented the 19-year gap), and chronic diseases exists — but uniform, timely annual reporting with trend analysis is not operational.

Direct connection: Section 2 of this report establishes, using Statistics Canada 2023 data, that the First Nations-Alberta life expectancy gap is approximately 19 years (62.81 vs 81.88 years). Call 19 directs publication of annual progress reports on exactly this category of indicator. The gap itself is the primary evidence of Call 19 implementation failure.

Call 20: Jurisdictional responsibility for off-reserve Aboriginal peoples

Status: Call 20 addresses the jurisdictional disputes covering Aboriginal people not resident on reserve — the same jurisdictional fracture Section 3 of this report documented in detail. The federal government has implemented certain targeted programs but has not produced a structural framework resolving the off-reserve responsibility question. Jordan's Principle (Section 8) was the primary mechanism for bridging the gap; its February 2025 operational bulletin narrowed the bridge.

Direct connection: The majority of Indigenous Albertans live off reserve. The primary care collapse documented in Section 7 affects them directly. The 72 percent Indigenous child welfare caseload documented in Section 9 is predominantly off-reserve. The 10.2-fold incarceration overrepresentation documented in Section 10 is substantially off-reserve. Call 20's requirement that the federal government recognize and address the distinct health needs of off-reserve Aboriginal peoples remains unmet at the scale required to affect these outcomes.

Call 21: Sustainable funding for Aboriginal healing centres

Status: The Government of Canada has provided targeted funding for specific healing centres. The Aqqusariaq (Nunavut Recovery Centre), funded in response to the Qikiqtani Truth Commission and Call 21, is an example of implementation. Sustainable, system-wide funding for Aboriginal healing centres addressing the harms of residential schools has not been delivered at the scale Call 21 directed. Indigenous-led healing infrastructure remains underfunded, as the Yellowhead Institute September 2025 review documented.

Direct connection: The Alberta Disability System Breakdown main report series documented the closure of the Family and Community Resource Centre (FRCCA) effective April 1, 2026, along with the elimination of individual AISH caseworkers and the defunding of PDD-related support organizations. These closures and defundings operate against the spirit of Call 21, which directs sustainable funding for healing infrastructure serving the Indigenous population.

Call 22: Integration of Aboriginal healing practices into Canadian healthcare

Status: Some Canadian healthcare institutions — including London Health Sciences Centre in Ontario and multiple regional health authorities — have formal memoranda of understanding with Indigenous friendship centres and traditional healers. These arrangements remain institution-specific rather than system-wide. In Alberta, Alberta Health Services has certain cultural safety initiatives but has not operationalized system-wide integration of traditional Indigenous medicine and ceremonial support in the way Call 22 directs.

Direct connection: For disabled Indigenous Albertans, traditional Indigenous healing practices are a category of care explicitly named in Call 22 that neither NIHB nor AHS comprehensively cover. The ADAP Health Benefits framework, like the AISH Blue Cross framework that preceded it, does not include coverage for traditional healing services.

Call 23: Aboriginal healthcare workforce

Status: All three sub-calls of Call 23 (number of Aboriginal healthcare professionals, retention in Aboriginal communities, cultural competency training) remain partially implemented. The Statistics Canada September 23, 2025 release documented that, in 2024, 24 percent of First Nations people living off reserve, 18 percent of Métis, and 23 percent of Inuit reported experiencing some form of unfair treatment, racism, or discrimination from a health care professional in the previous 12 months. The cultural competency training element of Call 23(iii) is therefore, by the lived experience measure Statistics Canada documented, not fully delivered.

Call 24: Medical and nursing school curriculum requirements

Status: Multiple Canadian medical schools have integrated Indigenous health curriculum elements. The University of Calgary Cumming School of Medicine's Indigenous Health Dialogue Working Group has developed a critical reflective framework for Call 24 implementation (documented in the peer-reviewed Academic Medicine literature, 2023). Inconsistent

implementation across institutions remains the documented finding of the September 2025 Yellowhead ten-year review.

Direct connection: The 650,000 Albertans without family physicians documented in Section 7 will, when they eventually access a physician, increasingly interact with a physician whose training did not embed Indigenous health, UNDRIP, Treaties, or anti-racism in the way Call 24 required. For the Indigenous disabled Albertan specifically, the cumulative effect is that when healthcare access is finally obtained, the provider is statistically likely to have had partial rather than comprehensive Call 24-compliant training.

Part 4: What the numbers look like ten years after the Calls

Statistics Canada 2024 data

The Statistics Canada report of September 23, 2025, titled “Indigenous health and the call for reconciliation in Canada,” provides the most recent comprehensive federal data on the health outcomes Call 19 directed the federal government to publish annual progress reports on. The 2024 findings:

- 24 percent of First Nations people living off reserve, 18 percent of Métis, and 23 percent of Inuit reported experiencing unfair treatment, racism, or discrimination from a health care professional in the previous 12 months.
- A majority of First Nations people living off reserve (86 percent), Métis (70 percent), and Inuit (82 percent) stated that it is important for health services to support Indigenous traditional medicines, healing and wellness practices — the direct subject of Call 22.
- 40 percent of Inuit, 18 percent of First Nations people living off reserve, and 16 percent of Métis reported travelling outside their communities for health care in the previous 12 months. Among Inuit who travelled, over half travelled more than 1,500 kilometres for services.
- Approximately half of First Nations people living off reserve (50 percent), Métis (52 percent), and Inuit (57 percent) said they were satisfied with wait times. Over one-quarter of Indigenous people reported unmet health care needs. Among First Nations women specifically, unmet needs rose to 38.6 percent.

These are not historical figures. They are the federal government’s own statistical agency’s documentation of the conditions the Calls to Action 18 through 24 were written to address, measured in the tenth year since the Calls were issued. The documentation confirms that anti-Indigenous discrimination in healthcare (Calls 22 and 23) is ongoing at rates approaching one in four off-reserve First Nations patients per year. That Indigenous demand for traditional healing practice integration (Call 22) is at eighty-plus percent. That geographic access barriers (Call 20) are substantial, with Inuit travelling more than 1,500 kilometres for routine care. That unmet health needs (Call 19) are documented at more than one in four Indigenous people, with First Nations women at nearly forty percent.

Ten years after the Truth and Reconciliation Commission's Calls to Action, the Government of Canada's own statistical agency documents that nearly one in four off-reserve First Nations patients reported experiencing racism from a health care professional in the past twelve months, that eighty-six percent want Indigenous traditional medicine integrated into their care, and that nearly forty percent of First Nations women report unmet health needs. The Calls were directed. The reality they directed governments to change has not materially changed."

Part 5: What the Calls mean for disabled Indigenous Albertans specifically

The Calls read against the ADAP transition

For a disabled Indigenous Albertan in April 2026, the seven health Calls of the Truth and Reconciliation Commission establish a binding moral framework — accepted in principle by the Government of Alberta on Call 18's first part, accepted in principle by the Government of Canada on all seven Calls — whose application to the July 1, 2026 ADAP transition produces the following findings:

- Call 18 establishes that the Crown must recognize and implement health-care rights under international law, constitutional law, and Treaties. The ADAP transition reduces provincial disability income and benefits without engagement with those Treaty, constitutional, and international rights — and without the consultation UNDRIP Articles 19 and 21 (Section 12) require.
- Call 19 directs measurement and annual reporting on indicators including life expectancy and chronic disease incidence. Section 2 of this report documented the 19-year life expectancy gap — the summary measure of Call 19 implementation failure.
- Call 20 directs the federal government to recognize and address the distinct health needs of off-reserve Aboriginal peoples. The majority of Indigenous Albertans on AISH are off-reserve. Their health needs are the precise population Call 20 identifies.
- Call 21 directs sustainable funding for Indigenous-led healing infrastructure. Alberta's defunding of related supports (PDD advocacy organizations, FRCCA closure April 1 2026, elimination of individual caseworkers) operates against Call 21's direction.
- Call 22 directs the integration of Indigenous healing practices. The ADAP Health Benefits framework does not include coverage for traditional healing services that 86 percent of off-reserve First Nations Albertans indicate they want integrated into their care.
- Call 23 directs increased Indigenous healthcare workforce representation and cultural competency training. The Alberta primary care collapse of 887 to 164 accepting practices

compounds this Call's implementation gap — there is no meaningful workforce reform in a system losing its overall workforce.

- Call 24 directs medical and nursing curricula to embed Indigenous health, UNDRIP, Treaties, and anti-racism training. The Indigenous disabled Albertan who eventually obtains a physician will, under current conditions, access one whose training was partially but not comprehensively Call 24-aligned.

The cumulative framework

Any single one of these Calls, fully implemented, would substantially improve the conditions this report has documented across fifteen preceding sections. All seven Calls, fully implemented, would constitute the health reconciliation the Truth and Reconciliation Commission considered necessary. At ten years, the implementation is partial. The Yellowhead Institute has characterized it as performative. The Statistics Canada data confirms that outcomes have not materially changed. The July 1, 2026 ADAP transition is scheduled to proceed in this context.

What this section establishes for the remainder of the report

The Truth and Reconciliation Commission of Canada's Calls to Action 18 through 24 constitute the specific, named, numbered health reconciliation framework that the federal and provincial governments of Canada have formally accepted as the measure of their reconciliation progress. Ten years after their issuance, the Yellowhead Institute's September 2025 review characterized Canadian health system implementation as "performative reconciliation." The Statistics Canada 2024 data confirms that the outcome measures the Calls addressed have not materially improved. For Alberta specifically, partial acknowledgment of Call 18's first part coexists with operational non-implementation across all seven Calls in the specific context of the disabled Indigenous Albertan population.

The July 1, 2026 ADAP transition is scheduled to proceed in tension with the spirit of Calls 18, 20, 21, 22, and 23 — and with the consultation obligations of Call 19's requirement for engagement with Aboriginal peoples. The transition is not merely disability income policy. It is a health reconciliation issue. The Calls to Action that Canadian governments have pledged to implement direct the opposite of what the transition will deliver.

Section 15, the final section of this report, will synthesize all fourteen preceding evidentiary sections into the specific question the report was written to address: what the July 1, 2026 ADAP transition means for the disabled Indigenous Albertan population, and what must happen to prevent the harm it will produce. Section 15 is the closing argument. It makes no new evidentiary claims. It integrates the claims this report has already established into the specific ask this report places on federal, provincial, and Indigenous governance actors.

Sources cited in Section 14

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